Barcode No	**86317933**	Lab No	16952405240001
Patient Name	**Mrs.MEETU BHAGAT**	Reg Date	24/May/2024 02:55PM
Age/Sex	30 YRS/Female	Sample Coll. Date	24/May/2024 01:58 PM
Refered By	RAINA CLINICAL LAB	Sample Rec.Date	25/May/2024 07:37 AM
		Report Date	25/May/2024 08:57AM

HAEMATOLOGY

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Hemoglobin HPLC			
Hb F(Foetal Hb) Level	1.8	%	< 2.0
Peak 2	3.2	%	
Peak 3	3.8	%	<10
Hb A0	82.4	%	
HbA2	2.5	%	1.9 - 3.5
Opinion	Normal Pattern		
Advise	Please Correlate Clinically		

Comment:
Haemoglobinopathy is a kind of genetic defect that results in abnormal structure of one of the globin chains of the hemoglobin molecule. Hemoglobin variants are abnormal forms of hemoglobin.

- All results have to be correlated with age and history of blood transfusion. If there is history of blood transfusion in last 3 months, repeat testing after 3 months from last date of transfusion is recommended.
- In case of hemoglobinopathy, parents or family studies, DNA Analysis and Genetic counseling is advised.
- **P3 window**- Above 10% is often indicative of either denatured forms of hemoglobin (degenerated sample) or may suggest a possibility of abnormal hemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
- **P2 window**- Above 10% is indicative of either glycated hemoglobin requiring correlation with diabetic status or may suggest a possibility of abnormal hemoglobin variant requiring further DNA studies for confirmation.
- Screening of the spouse are strongly recommended, if pregnant woman have hemoglobinopathy.
- This test detects Beta thalassemia and hemoglobinopathies. DNA analysis is recommended to rule out alpha thalassemia and silent carriers.
- Mild to moderate increase in **fetal hemoglobin (HbF)** can be seen in some acquired conditions like Pregnancy, Megaloblastic anemia, Thyrotoxicosis, Hypoxic conditions, Chronic kidney disease, Recovering marrow, MDS, Aplastic anemia, PNH, Medications (Hydroxyurea, Erythropoietin, Butyrates) etc.
- Iron deficiency Anemia may be associated with **low HbA2** result and may mask thalassemia condition also.
- **Elevated levels of HbA2** are seen in Beta-Thalassemia, and also in other conditions like Sickle cell Alpha-Thalassemia, Megaloblastic Anemia, Unstable Hemoglobinopathies, Zidovudine-treated HIV patients, Congenital Dyserythropoietic Anemia (CDA) type 1 and Hyperthyroidism.***Results relate only to the sample, as received.**

Prashant
Dr Prashant Goyal
(Chief Pathologist)

Chromatogram Report

Tosoh HLC723G11 V3.07 1 2024-05-25 08:33:30

ID 86317933
Sample No. 2024052508140040 SL 0004 - 02
Patient ID
Name
Comment

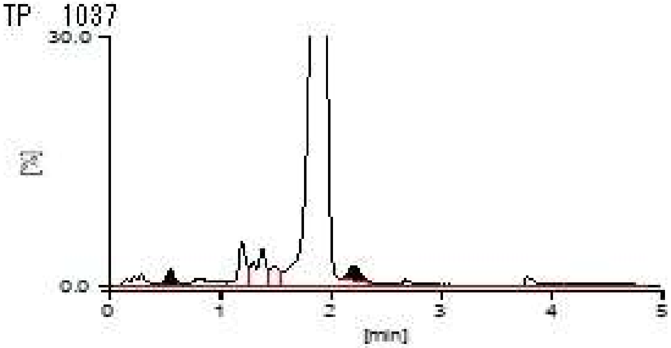
CALIB F $Y = 1.2706X + 0.2676$
A2 $Y = 1.4475X + 0.3135$

Name	%	Time	Area
F	1.8	0.55	23.42
A0	82.4	1.91	1572.68
A2	2.5	2.22	30.48
E+			
D+			
S+			
C+			

Total Area 1907.71

HbF 1.8 % **HbA2 2.5 %**

TP 1037



[Unknown Peak]

Name	%	Time	Area
P00	1.7	0.29	32.18
P01	0.0	0.47	0.61
P02	0.2	0.63	2.93
P03	1.4	0.80	27.50
P04	3.2	1.20	60.57
P05	3.8	1.38	71.98
P06	1.7	1.49	32.85